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June 16, 2025

Mehmet Oz, MD
Administrator
Center for Medicare and Medicaid Services
U.S. Department of Health and Human Services
7500 Security Blvd.
Baltimore MD 21244-1850

Thomas Keane, MD
Assistant Secretary for Technology Policy
U. S. Department of Health and Human Services
200 Independence Ave SW
Washington DC 20201

Re: Request for Information; Health Technology Ecosystem; RIN 0938–AV68

By electronic submission

Dear Administrator Oz and Assistant Secretary Keane:

Omada Health, Inc. ("Omada") hereby submits the following comments in response to the above referenced Request for Information (RFI).

1. Executive Summary

This RFI could not be more timely or necessary. Sixteen years after the HITECH Act (HITECH) and its parent law, the American Recovery and Reinvestment Act (ARRA), unleashed a flood of digital health innovation, the time has come to assess how to effectively make that innovation available to America's seniors, 79% of whom use a smartphone regularly.[1] Of course, HITECH resulted in 90% of physician's offices and nearly 100% of hospitals using Certified EHRs ("CEHR"). It also, however, fostered the development of new modalities of care based on advances in computing, standards for digital health data (semantic and use standards), and patient empowerment. Omada has also delivered innovation that ARRA in part made possible. As a result, Omada has delivered its virtual, digitally-enabled health care services to over one million Americans since its founding. This includes serving thousands of Medicare beneficiaries as a supplemental benefit through such partners as Intermountain Health, Blue Cross Blue Shield of Minnesota, and Security Health Plan.

The RFI asks detailed and incisive questions about making "digital health tools" and "digital products" (as defined in the RFI) available to America's seniors. As discussed in greater detail

[1] Pew Research Center, November 2024, https://www.pewresearch.org/internet/fact-sheet/mobile/

below, however, we believe the RFI should focus not on the digital health tools or products. Rather, it should focus on the healthcare services that digital technology makes possible. Instead of asking about technology, we believe the RFI should ask: "*What policies could help ensure that Medicare beneficiaries have access to the same efficacious digital healthcare services that Americans under 65 have and can use everyday?*"

We believe that, where care is effective, safe, high-quality, and accessible, it should be included among all the modalities of care available in Medicare. When this occurs, Medicare beneficiaries will have the same choices of where and how to get their health care services as do people under 65.

It is not the lack of prescriptive regulation of technology that is preventing seniors from benefiting from healthcare services that use digital health tools or products. Nor is it a lack of incentive payments, like MACRA, to take up digital health tools. Rather, we believe that what is preventing the uptake by seniors of care delivered with the help of digital health tools and products is *coverage for that care* by the Medicare program. When asynchronous care that meets other relevant standards for safety, quality, accreditation is covered by Medicare, competition and the fee markets will result in seniors using those services when they want to use them. These beliefs shape our answers to the RFI questions.

Part II gives background on Omada Health and illustrates how care can, and is, effectively delivered to people over 65 by human professionals connecting with patients using "digital health tools" and "products." Relevant illustrations from our mobile apps are included on Appendix A. We describe Omada's health care services in detail so that, when you read Part III, you will have sufficient factual context.

Part III answers specific questions where Omada can provide helpful information. (We do not answer all RFI questions.) Within Part III, we have reproduced only the RFI questions that we are answering.

Part II: Background on Omada.

Founding and Services: Omada is a 14-year-old digital health company that provides care-team-based health care services for the prevention of diabetes and other cardiovascular diseases, for the management of type 1 and type 2 diabetes and hypertension (whether as single conditions or comorbid). Since 2020, we also offer video-based tele-physical therapy using licensed physical therapists. In 2024, we also launched a care track of our lifestyle behavior change programs designed specifically to support individuals taking GLP1s for weight loss.

Member Demographics, Including Medicare: Omada has served over 1 million adults since its founding, and we estimate that 20 million adults in the U.S. have benefits coverage for one or

more Omada programs through their employers (including retiree medical plans), health plans, pharmacy benefit managers, health systems, or other customers. An estimated 3.2% of our cumulative membership is over age 65, or approximately 32,000 individuals cumulative since 2013. Some of these members are people over 65 who are working, but a significant number are or have been Medicare beneficiaries. Our coverage for members receiving Medicare comes from our contracts with a number of Medicare Advantage plans, as a supplemental benefit. The balance of our members (more than 975,000) have coverage for Omada through commercial insurance or their employer's ERISA plan. For these hundreds of thousands of individuals, Omada has submitted many hundreds of thousands of claims for payment as a health care provider.[2]

Unfortunately, current CMS regulations preclude Omada from serving Medicare beneficiaries as a Medicare supplier and not as a supplemental benefit or billing Medicare directly. In fact, our status as a "supplemental benefit" with our Medicare Advantage plan customers is a direct byproduct of CMS rules that prohibit suppliers of asynchronous care from being Medicare suppliers. This prohibition prevents companies like Omada from participating in Medicare on a fee-for-service basis and also chills many Medicare Advantage plans from contracting with us because we do not have the familiar Medicare supplier status.

Our Accreditations and Quality Recognitions: Omada is fully recognized by the CDC's DPRP program for our diabetes prevention program. The Academy of Diabetes Care and Education Specialists accredits Omada's diabetes education programs using the same standards it applies when accrediting in-person programs under 42 USC 1395x(s)(2)(S) and (qq). In May 2023, Omada renewed its NCQA's Population Health accreditation for its diabetes program for an additional three years. Omada's tele-physical therapy services are URAC-accredited for telehealth.

Our Participation in Nationwide Health Information Exchange: Salient for this RFI, Omada has two electronic health records that meet the definition of Section 13400(5) of American Recover and Restoration Act (ARRA)[3] but has chosen not to use an available "certified electronic health records" system because available systems lack features that Omada needs for its professionals and the flexibility and ability for Omada to modify the EHR as its business evolves. Omada does, however, exchange health information, including protected health information (PHI), regularly with health systems (through contracts that enable Omada to participate in CareQuality) and with payers (through secure exchange of CSV files, for example).

How Omada Operates

[2] 45 CFR160.103
[3] 42 USC 17921

Omada is a digital provider, and accordingly, there are no clipboards or long waits on the phone just to make an appointment. More details on our digital operations are included below. You can also watch demo videos of Omada works here.

Eligibility and Outreach: Omada contracts with its payer customers as a "health care" "provider" and "covered entity" as defined in 45 CFR 160.103. Once a contract is complete, we reach out to the payer's population by email or direct mail, at the payer's request, to let the population know that they have coverage for Omada. If an individual is interested, they are invited to click through to a secure, HIPAA-compliant web page that we own and operate. There, individuals can check whether they have coverage,[4] and confirm whether our programs are clinically appropriate for them. If our programs are right for the individual, they are accepted and invited to download our app, set up their accounts, and provide some basic health history.

Cardiometabolic Programs: For members in our programs that prevent and provide care for cardiometabolic disease, we next provide the individual with the relevant digital health tools that are inherent in our program. (See Table 2 below.)[5] All of these tools, except continuous glucose monitor sensors (CGMs), where appropriate, are shipped automatically. If a member's benefits include coverage for a CGM, we will ship the CGM once our contracted pharmacy receives an appropriate prescription, and we can also facilitate an evaluation for such a prescription with a telemedicine partner.

The first time a member uses these devices, the devices connect securely to Omada data systems, primarily through cellular technology, so that members don't have to struggle with syncing bluetooth or wifi.[6] Our cellular technology also means these devices work in nearly every location in the U.S., including for example rural communities in Alaska. We handle the secure transmission of the collected data so our members don't have to worry.

Each member in our cardiometabolic programs is matched to a care team, assigned a peer group, and can then start their program participation. Care teams consist of real human professionals with different scopes of practice, as shown in Table 1 below.

Table 1: Omada Care Team Composition

Program	Lifestyle Coach	CDCES/ Diabetes Educator	Credentialed Exercise Specialist	LCSW as internal expert for the Care Team	Licensed Physical Therapist
Omada for Prevention and Weight Loss	Y	N	N	Y	N

[4] We conduct eligibility checks using HIPAA 270/271 transactions, just like other health care providers.
[5] Digital tools are manufactured by third parties pursuant to supply agreements and provided for Omada members.
[6] CGM data is synced to the CGM manufacturer's data systems through an in-app experience where the member specifically decides to connect Omada's app to the CGM Manufacturers data, based on the provisions of section 13504(e) of HITECH. (42 USC 17935)

Omada for Diabetes	Y	Y	N	Y	N
Omada for Hypertension	Y	Y	N	Y	N
Omada GLP-1 Care Track	Y	Y	Y	Y	N
Omada for MSK	N	N	N	Y	Y

Once the program starts, the members and their care teams communicate asynchronously through our secure app. Members may also engage with peer groups for support from within our app. Because we are a health care provider and a covered entity under HIPAA, all data that is collected from or about a member is protected health information (PHI).

For our more acute programs, like those for people with diabetes or for those taking a GLP-1 for weight loss, Omada participates in nationwide health information exchanges, through agreements that allow us to participate in CareQuality, to receive prescription medication history.

In addition to the education we provide through our mobile apps, we supply the digital tools in Table 2, manufactured by third parties, which provide real-time clinical data to the members and their care teams. The member can see this information in the mobile app, and the care team sees it in our EHR. The equipment comes included in the price of the Omada program paid by the payer, so individuals do not receive surprise bills from Omada.

Table 2: Cardiometabolic Devices

Scale (FDA Class 1)	Blood Glucose Monitor (FDA Class 2)	Blood Pressure Monitor (FDA Class 2)	CGM Sensors (FDA Class 3, RX required)
Y	N	N	N
Y	n/a	n/a	n/a
Y	Y	N	Y
Y*	Y	N	Y
Y*	N	Y	N
Y*	Y	Y	Y

Members have access to a substantial library of educational content on diet, exercise, nutrition, stress, and other topics unique to people with diabetes (like traveling with insulin), people with hypertension, or those taking GLP-1s. In May, 2025, Omada also added nutritional education features powered by AI, which are available 24/7 to help members understand whether their food choices are nutritious. Members also have access to a wide variety of

self-help behavioral health tools that have been shown to help effectively address mild to moderate symptoms of anxiety or depression.[7]

As members, individuals can access real-time data through their Omada mobile applications in a simple, elegant, easy-to-understand user interface. At any time, an individual can also share Omada results with others (e.g., a PCP, a family member, a care giver).

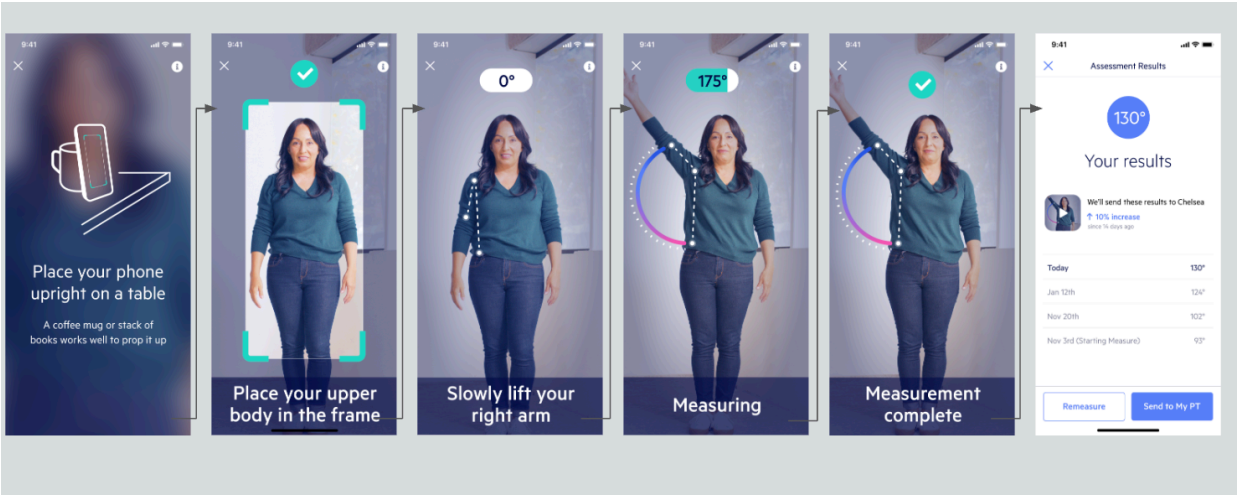
Appendix A includes sample screenshots that illustrate a number of features, including communications with an Omada care team, establishing connections between the Omada mobile app and systems of the CGM manufacturer, interfaces for tracking glucose and blood pressure, and processes for using our self-help behavioral health tools. To see what members think, refer to this video of a member and their coach, or this video of how easy our program is to use.

Omada bills for its cardiometabolic services using a unique, outcomes-focused formula we call "activity-based billing." When an Omada member uses our program (ranging from messaging their coach to tracking their food) a designated number of times in ways that evidence shows can improve the likelihood of a successful outcome, this triggers a claim for our services to the payer. If an individual does not use our program, we do not bill other than for the initial set up and equipment shipping.

Physical Therapy Programs: For our musculoskeletal services, Omada also contracts with insurance companies and ERISA plans as a health care provider. When an individual applies to receive physical therapy from Omada, they download the relevant Omada mobile app, complete their health history online (again, secured in accordance with HIPAA) and, where appropriate, request an appointment. Members can typically schedule that appointment within 24 hours on average with a licensed physical therapist, contracted through an appropriately structured professional corporation and licenses in the appropriate jurisdiction where the individual lives.

Unlike our cardiometabolic services, for many members, our physical therapy (or PT) services start with a 30-minute synchronous video appointment where the licensed physical therapist uses live conversation and AI-assisted tools, such as computer vision, to evaluate the patient's needs. See Illustration 1 below. Our protocols also include that, where, in the physical therapist's judgment, virtual physical therapy is not appropriate for the patient, the patient will be referred to an in-person PT or other appropriate provider, such an orthopedist. Physical therapy courses of treatment last four to ten weeks on average, and the course of treatment is a bundled service. Omada bills the payer a flat rate for the bundled service with a CPT-coded claim and collects deductibles, when applicable, just like with in-person physical therapy, but without the weeks-long waits.

[7] Naqvi, J. B. PhD; Napoleone, J. M.,PhD, MPH; Devaraj, S. M. PhD; Kaperick, C. LCSW; Linke, S., PhD, *Improvement in depression and anxiety symptoms among participants in a virtual cardiometabolic condition management program.* Presented at the 46th Annual Society of Behavioral Medicine Meeting and Scientific Sessions. March 28, 2025, whitepaper at: https://resourcecenter.omadahealth.com/for-individuals-diabetes/anthony-diabetes-ease-of-use.



Part III: Detailed Comments

As mentioned above, Omada has prepared responses only on questions relevant to Omada's health care services and experience as a provider of asynchronous health care. Below, **bold-face** text is from the RFI.

1. Patient Needs

PC–1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

In response to PC-1, please see the descriptions of our programs above and the videos linked above and the illustrations are in Appendix A. In sum, we believe our members love our combination of human care teams with real-time health tracking (for personal accountability). Omada has designed its programs specifically to take advantage of digital technology to supply real-time, real-world health information so that care teams and patients could collaborate effectively.

Here is what one Medicare Advantage member receiving our cardiometabolic services recently said about Omada:

> *I was excited to learn that my [redacted plan name] Medicare Advantage Plan offered the Omada Health program. In the past, I was considered to be pre-diabetic, but at my most recent check-up, my blood work indicated no diabetes or pre-diabetes. I want to keep it that way, and the Omada Health program provides a good "back-stop" for me. I'm confident that the program's accountability components (tracking my weekly goals, activity, food, and weight) is what I need to stay on track. I appreciate the gentle*

coaching and the feedback I have received. Thanks [to my plan] for making this available through the Medicare Advantage option. Jane, Age 73

PC–5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

We do not believe additional oversight of the quality of the digital technology is necessary to foster its use by Medicare beneficiaries. Existing private-sector accreditation programs can demonstrate where services are high-quality, private, secure, and effective, and Omada has leveraged these programs to assure its prospective customers and prospective members. All of the accreditation organizations listed below are available today to organizations that supply asynchronous health care services like Omada's and that use the digital tools and digital products to support those services.

- ***Academy of Diabetes Care and Education Specialists*** accredits Omada's Diabetes Self Management & Training program (DSMT). This organization is also authorized by 42 USC 1395x to accredit DSMT programs that bill Medicare on a fee-for-service basis.
- ***National Committee for Quality Assurance*****:** We believe Omada was the first (in 2021) and perhaps the only 100% virtual health care provider to be accredited by NCQA for NCQA's Population Health accreditation. We also understand that NCQA is evolving its accreditation program regularly to meet the needs of an ever-more-digital healthcare ecosystem
- ***URAC Telehealth***: Omada's physical therapy program is accredited by URAC
- ***Centers for Disease Control Diabetes Prevention Recognition Program***: Certain deployments of Omada's prevention program have been fully recognized by CDC since 2017.

In addition, providers of health care services who use digital health tools or build digital health products in support of their health care services are subject to the following additional standards, among others, for quality, safety, privacy, and security:

- HIPAA rules for privacy, security, and claims billing;
- State medical confidentiality rules applicable to providers of healthcare;
- State and federal program-integrity rules;
- State laws for quality, scope of practice, and malpractice liability;

- FDA rules for accuracy and reliability of devices, backstopped by Quality Management Systems; and
- FTC rules for privacy, security, and accuracy of representations.

In addition, in the U.S., the vast majority of care outside of Medicare is paid for by health insurance companies, whose conduct is also heavily regulated. Accordingly, a digital health company such as Omada will also be subject to contractual requirements for safety, quality, access, etc. that flow down from insurance companies.

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

See our answer PC-8(c) re TEFCA, below.

PC–6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

We noted above that 79% of U.S. residents over the age of 65 use smartphones regularly, which we believe is a high number. At this depth of usage, it is likely that most seniors and their caregivers effectively use smart phones. This means that understanding app-based technology is likely not a barrier to the uptake of digital health care tools and products for seniors. Individuals over age 65 who have access to Omada's services use them in the same ways that people under the age 65 use them, and in fact, some of our research demonstrates that Omada members of Medicare age use our services with greater engagement than their under-65 counterparts. For example, one randomized controlled trial of Omada for Prevention compared to traditional care found that the sub-population of Medicare-age beneficiaries who received the intervention lost more weight and engaged with the Omada program more frequently than their under-65 counterparts.[8] We have also seen similar results when looking at seniors using our tele-physical therapy services compared to the under-65 population using that service.[9]

Based on our experience, we believe that seniors, if given access to well-designed, clinically sound, asynchronous, virtual health care services, would willingly use them. We also believe, therefore, that it is not the technology itself or an absence of more regulation that holds seniors back but rather an absence of coverage as a Medicare benefit.

PC–7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

[8] Michaud, T., et al,Cost and cost-effectiveness analysis of a digital diabetes prevention program: results from the PREDICTS trial, https://pubmed.ncbi.nlm.nih.gov/36809348/

[9] Mulcahy, J., DPT, et al, Defying Stereotypes: Older Adults as High Engagers in App-Based Telehealth Physical Therapy, *Topics in Geriatric Rehabilitation* 39(4):p 307-311, October/December 2023. | *DOI:* 10.1097/TGR.0000000000000414

Omada believes the best way to collect this data would be to enable healthcare providers who use digital health tools and digital health products to submit claims for that care. If this were to occur, digital care, like traditional care today, would produce substantive clinical data that can be used to further study digital care and its efficacy, safety, and economics (both immediate cost and long-term ROI), just as we study the claims data of traditional care today.

Unfortunately, there are current impediments to this analysis. For example:

1. A significant majority of CPT codes require synchronous care or are not otherwise amenable to use by providers of asynchronous care. In addition, there are even more Category 1 E&M codes that describe care that can safely and effectively be supplied asynchronously with the support of digital health tools and products but cannot be billed for using existing CPT codes. Some research has been conducted on the quantity of Category 1 E&M CPT codes for services that could be supplied asynchronously as well as synchronously.[10]

2. Not all digital health tools and digital health products (and perhaps even very few) are FDA-regulated devices, and many, if not most, should *not* be treated as devices under the provisions of the 21st Century Cures Act. This means, however, that category 2 CPT codes (for devices) are not usable for many health care *services*. Using device codes would muddy the analytic data about what care *services* actually occurred and could undermine the integrity of results on outcomes or costs.

If appropriate codes could be billed by professionals who supply health care with the assistance of digital tools or products, the claims data could be used to more precisely evaluate the impact of that care. For example, with claims data and NPIs, we can evaluate differences in the calibre of care between physicians and other licensed professionals working at the top of their license. This, in turn, could yield informative evidence on effective ways to bend the cost curve in our healthcare system.

2. Data Access and Integration

PC–8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

> **c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?**

As Omada also mentioned in its comments on the HIT-2 proposed rule, because TEFCA establishes a two-tiered system for responses to queries, digital health care providers are at significant disadvantage. The top-tier providers under these rules include those that benefited

[10] Duffy, S., and Jain, S., MD, "*To Integrate Virtual Care, Start By Redefining The 'Visit*,' Health Affairs Forefront, July 26, 2022

from meaningful use and other human providers licensed by states to provide professional services. Queries from these providers must be responded to by other exchange participants. All other healthcare providers fall into a second tier. This tier includes the care that Omada supplies through human care professionals, including those who are accredited (and in two states, licensed) to provide diabetes education without direct physician supervision. It also includes durable medical equipment suppliers and other entities who are considered providers under the same HIPAA data-handling rules that apply to hospitals and physicians but who are not entitled to the same presumptions of query legitimacy as physician providers or hospitals. A copy of Omada's prior comments on this is attached as Appendix B. We believe this two-tiered system is far removed from the nationwide exchange's standard of "without special effort" described in 21st C. Cures Act.[11]

PC–10. How is the Trusted Exchange Framework and Common Agreement (TEFCATM) currently helping to advance patient access to health information in the real world?

As discussed in our answer to PC-8, we are not sure that TEFCA is particularly helpful for companies that provide health care services with the support of digital health tools or digital health products where their professionals are not in scope for the top-tier status under TEFCA rules.

a. Please provide specific examples.

We refer to two examples:

First, prior to the Sequoia Project's finalizing rules that created this two-tiered system, we observed friction around the legitimacy of health care services like Omada's, merely because organizations like Omada may choose not to use certified EHRs. At the same time, we feel there was little to no public dialogue about how care can and should work where communication occurs via mobile apps.

Second, in connection with that friction, there were moments when "Actors" (as defined in ONC Cures Act final rule of May 1, 2020 that were also "Developers" of "Health Information Technology" (both as defined in the Information Blocking rules) cut off access to information exchanges and thus rendered health information from companies like Omada inaccessible to some of their customers and health systems partners that leveraged those nationwide exchange channels. This was all eventually resolved, but the events were bad for patient care and inconsistent with statutes and regulations.

b. What changes would you suggest?

As Omada mentioned in its comments on HTI-2, we believe the simplest approach would be to treat all organizations that meet the HIPAA definition of a "healthcare" "provider" in the same way for purposes of health information exchanges. All health care providers who are

[11] Section 4002(a) 21st C. Cures Act, codified at *42 U.S.C. 300jj–11*

contracted to provide care and who are regulated as covered entities under HIPAA rules should receive query responses, even if the provider's form of communication is through a mobile app. We believe this approach would also address problems arising from organizations that are not health care providers, as the HIPAA term requires that the entity provide "health care" and bill electronically to a payer with a "claim" that meets HIPAA transactional standards.

C. Providers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve providers. While we certainly want providers to answer questions in this section (and in other sections) from the provider point of view, we also invite all stakeholders to provide their viewpoints on the provider workflows as appropriate. A trust community is an ecosystem of health networks that operate under a common set of rules and technical requirements to securely exchange electronic health information.

1. Digital Health Apps

PR–1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC–5) digital health products for their patients?

a. What are the current obstacles?

Omada has customer relationships with several health systems. We consistently hear from these health systems that they would like to encourage their members to use Omada services. However, because there is no reimbursement available through Medicare, the health systems cannot enable these services for their Medicare members who do not have Medicare Advantage coverage with a plan that contracts with Omada. Thus, we believe the current largest obstacle is a lack of coverage for the digital health care services.

b. What information should providers share with patients when using digital products in the provision of their care?

Omada is a health care provider and a covered entity with services based on well known care protocols, like diabetes prevention and standards of care supported by the American Diabetes Association. Because of this, we hear from many physicians and other primary care providers that they have confidence in our services. We believe that this type of reassuring information about legal frameworks, standards of care, and accreditations, like the aforementioned ADCES, NCQA, and URAC accreditations, is beneficial for patients.

c. What responsibilities do providers have when recommending use of a digital product by a patient?

All health care providers should meet minimum standards of care for any advice they provide, whether recommendations for referrals, lab tests, or mobile apps. We believe individual

providers will choose to make recommendations based on well recognized indicators of quality like accreditations, sound science, and safety records.

2. Data Exchange

PR–6. Is TEFCA currently helping to advance provider access to health information?

As discussed in our answer to PC-8, we are not sure that TEFCA is particularly helpful for companies that provide health care services with the support of digital health tools or digital health products where their professionals are not in scope for the top-tier status under TEFCA rules.

PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

Providers can exchange data when necessary for the discloser's or the receiver's treatment or health care operations needs, and Omada exchanges data for the same reasons. This was also true at the time that HITECH was enacted.

Accordingly, it may be more informative to ask: "*What can CMS or ASTP do to further align incentives across the healthcare information ecosystem to improve the quantity of nationwide exchange and the caliber of the data exchanged?*" When entities that provide health care services using digital health tools and products are viewed consistently with other providers of reimbursed healthcare, regardless of the modality, we believe market forces will drive incentives into alignment.

3. Digital Identity

PR–10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs):

As a fully digital program with a sophisticated security infrastructure designed to comply with the latest guidance from NIST and other relevant organizations, Omada does not find the concept of digital identity credentials difficult to understand or implement. We note, however, that the vast majority of the healthcare system operates in a very different physical and software environment, and others may not allocate sufficient resources to issues like digital identity.

4. Information Blocking

PR–13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

We note that 42 USC 300jj(3) includes "any other category of health care facility, entity, practitioner, or clinician determined appropriate by the Secretary." The Secretary's regulations state that all individuals and organizations who supply "health care" and bill with electronic claims are "providers" and "covered entities" for the purpose of keeping that electronic health information private and secure (45 CFR 160.103). In contrast, current ASTP regulations address only a subset of entities that the Secretary has deemed to be "providers." This misalignment in scope makes achieving nationwide health information exchange difficult. (Please see our response to PC-8, above.)

This misalignment can also make it difficult to integrate new, innovative modes of care that are built on digital health tools and products into the healthcare delivery system, especially for Medicare beneficiaries. Under the current system, only providers who use certified EHRs are effectively disincented from information blocking by ONC rules, and only providers who use CEHRT benefit from nationwide health information exchange rules (TEFCA) that require a response to a query. (All HIPAA covered entities are equally incented to provide patients with their own PHI.) We believe that these mismatches impede full nationwide health information exchange among all covered entity providers and that provider covered entities who use digital health tools in support of the care they supply are especially disadvantaged.

PR–14. How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

Omada's understanding is that more than 1,000 complaints have been submitted, per ASTP data. We believe it would be helpful for all stakeholders to understand the results of the relevant investigations so that stakeholders can learn what works and what behaviors are not allowed.

E. Technology Vendors, Data Providers, and Networks

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve technology vendors, data providers, and networks. While we certainly want technology vendors, data providers, and networks to answer questions in this section (and in other sections) from their point of view, we also invite all stakeholders to provide their viewpoints on the technology vendor, data provider, and network use cases as appropriate.

1. Ecosystem

TD–1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

As discussed above, Omada believes that predictable and appropriate reimbursement pathways for health care services that are delivered with the assistance of digital health tools or digital health products would spark competition. Once that occurs, we believe that beneficiary choice will be a helpful factor.

3. Technical Standards and Certification

TD–4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

We believe that competition and the free market will advance the use of open, standards-based APIs when the health care providers who use digital health tools and products (but not CEHRT) are seen as an equal and legitimate mode of care for Medicare beneficiaries.

TD–6. What unique interoperability functions does TEFCA perform?

We recognize that TEFCA is required by the 21st Century Cures Act and that TEFCA serves an important function in standardizing contracting terms and therefore potentially helping to reduce contracting time. To the extent, however, that TEFCA and its administrator (the Sequoia Project) establish rules that chill the innovative use of digital health tools and digital health products in ways that can improve the health of Medicare beneficiaries, we believe the entire structure may need to be reconsidered for improvement.

In addition, we observe that TEFCA's standard contracting terms do not account effectively for new modalities of providers. Moreover, because the Sequoia Project operates according to its own governance rules and not the Administrative Procedures Act, the Sequoia Project's process for iterating on its rules may not sufficiently account for organizations that supply health care services as providers using these new modes of care.

a. What existing alternatives should be considered?

We have mentioned two alternatives above: elimination of the two-tier system for responding to queries and revisions to treat all HIPAA "providers" of "health care" / "covered entities" in the same way.

TD–7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

We believe that USCDI has been helpful in instances where Omada has been able to successfully exchange data with traditional health care systems and providers but that it has

been less helpful in addressing the weaknesses already mentioned in the governance policies of the current nationwide information exchange system.

> **d. Given improvements in language models, would you prefer a nonproprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?**

We read TD-7.d as referring to large language models and the ability of generative AI systems to comprehend, summarize, and interpret unstructured data. We do believe that, in the very near future, AI will be widely used to understand unstructured data, we believe that the resulting interpretations will have high fidelity to the original data. Accordingly, federal policy should plan now for the ability of AI to reduce the need for semantic interoperability in structured data.

TD–12. Should CMS endorse non- CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Omada believes that this type of accreditation or endorsement is best left to the free market.

TD–13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

See TD-8 above. For example, the ability of AI to comprehend, summarize, and interpret will enable patients themselves to acquire their entire longitudinal records, to understand them, and to use that information as they wish: for second opinions, to pursue benefit appeals, in research, or even with their chosen malpractice attorney.

5. Compliance

TD–18. Information blocking:

> **c. Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?**
>
> See answer to PR-13, above.

F. Value-Based Care Organizations

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve value-based care organizations. While we certainly want value-based care organizations to answer questions in this section (and in other sections) from the value-based care provider point of view, we also invite all stakeholders to provide their viewpoints on the value-based care workflows as appropriate.

1. Digital Health Adoption

VB–1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

As discussed above, the biggest barrier that we observe to the adoption of health care services delivered with support from digital health tools and products is a lack of coverage for the use of those tools. This is true both for traditional providers, if they choose to supply such tools, and for companies like Omada who use these tools as providers in their own right.

VB–2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

As discussed above, we believe that, once there are clear reimbursement paths, competition and innovation will drive the development of appropriate services for Medicare beneficiaries that use these techniques. Omada currently uses population health analytics and risk stratification and maintains a keen desire to keep patients engaged when delivering its care. In the past two years, we have added new AI features that support our care teams with clinical decision support and other member-facing AI features that augment our patient education. As mentioned, due to coverage limitations, our services are not as widely available to seniors across the U.S. as they could be.

VB–3. What are essential health IT capabilities for value-based care arrangements?

As discussed in Section II, all of Omada's services are paid in a manner consistent with value-based care. Our tele-physical therapy is a fixed-fee, bundled service. Our cardiometabolic services are paid only as individuals engage with the program in a way that the payer and Omada agree is likely to improve clinical outcomes.

In that context, we believe that there are plenty of health IT capabilities that exist today that could be (and the case of Omada, already are) supportive of value-based care and that, when used appropriately, digital health tools and products can produce outcomes data that is informative, or even definitive, of whether value is being produced. However, when federal policy does not cover these types of health care services, the nation overall receives less value-based care.

We note that outcomes data is missing from the list at VB-3a. Here are a few examples of outcomes data based on current technology:

- Real-time glucose outcomes, including CGM-produced Glucose Metrics Indicators, showing whether glucose is controlled or even lowered, as is possible with the digital devices like those shipped by Omada.

- Gait, through gait-measurement features powered by gyroscopes in smart phones, usable before and after orthopedic surgery for knees or hips.[12]
- Digital deployment of standard mental health assessment instruments, such as PHQ4 or GAD 7. (Omada uses these tools within our app to assess depression and anxiety in our population.)

VB–4. What are the essential data types needed for successful participation in value-based care arrangements?

CMS states that "Value-based programs reward health care providers with incentive payments for the quality of care they give to people with Medicare." In that context, outcomes data is essential, including outcomes data that results from the use of digital health tools and digital health products when delivering health care services.

2. Compliance and Certification

As discussed above, Omada does not use ONC-certified technology. So, we have not answered VB-5 through VB-10. We look forward to further discussions about how best to expand the reach of nationwide health information exchange and to foster innovation in a world where digital health tools and products are ubiquitous and interoperable.

3. Technical Standards

VB–14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

To the extent that digital identity credentials improve trust, they are valuable, but only when all health care providers can take advantage of them, even if a particular provider does not use CEHRT. A digital identity credential, being crucial for trust and security, should be available to all types of health care provider covered entities, if they can meet applicable and fair standards.

IV. Conclusion

As one of the nation's most notable suppliers of asynchronous health care services with support from digital health tools and digital health products, Omada hopes the information supplied above has been helpful. We look forward to deeper conversations about creating reimbursement pathways for new modalities of care in Medicare and about what Omada, as an industry leader, can do to help CMS and ASTP bring innovative care to seniors.

Respectfully submitted.

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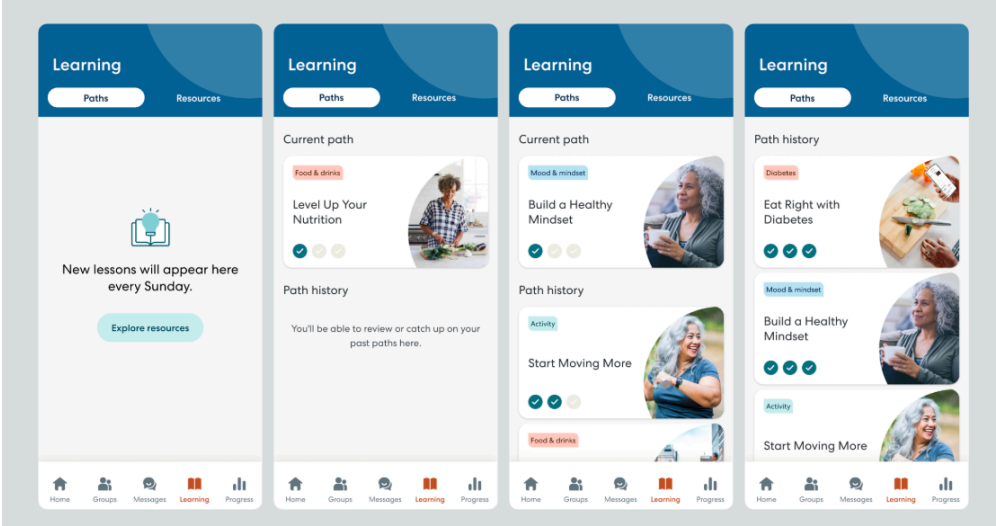
[12] Gowounda, D., et al, **A Low Cost Alternative to Monitor Human Gait Temporal Parameters–Wearable Wireless Gyroscope,** IEEE Sensors Journal >Volume: 16 Issue: 24, Dec. 15, 2016.

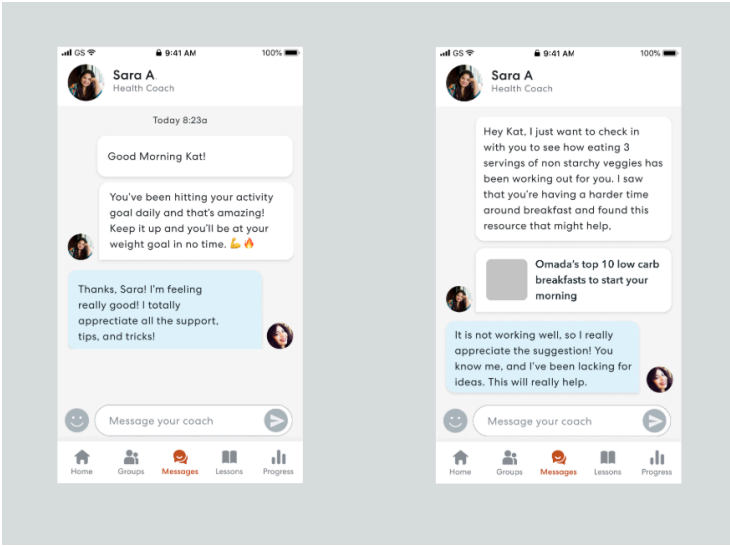
Lucia C. Savage
Chief Privacy & Regulatory Officer
Omada Health, Inc.

Appendix A, Omada Health Comments to CMS & ASTP June 16, 2025

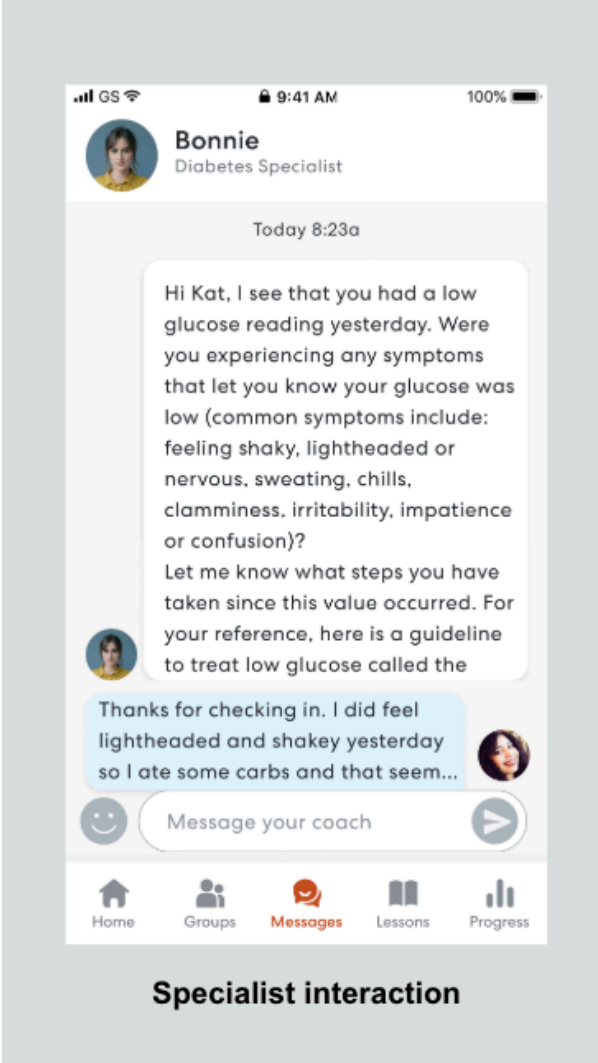
Accessing Omada Lessons



Messaging with a Health Coach

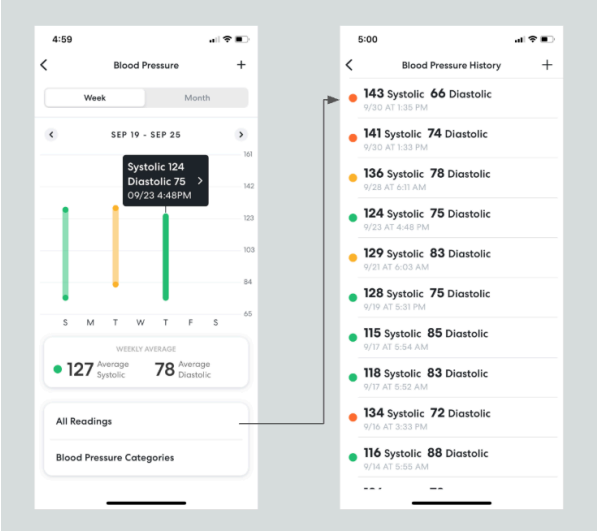


Messaging with a Clinical Specialist

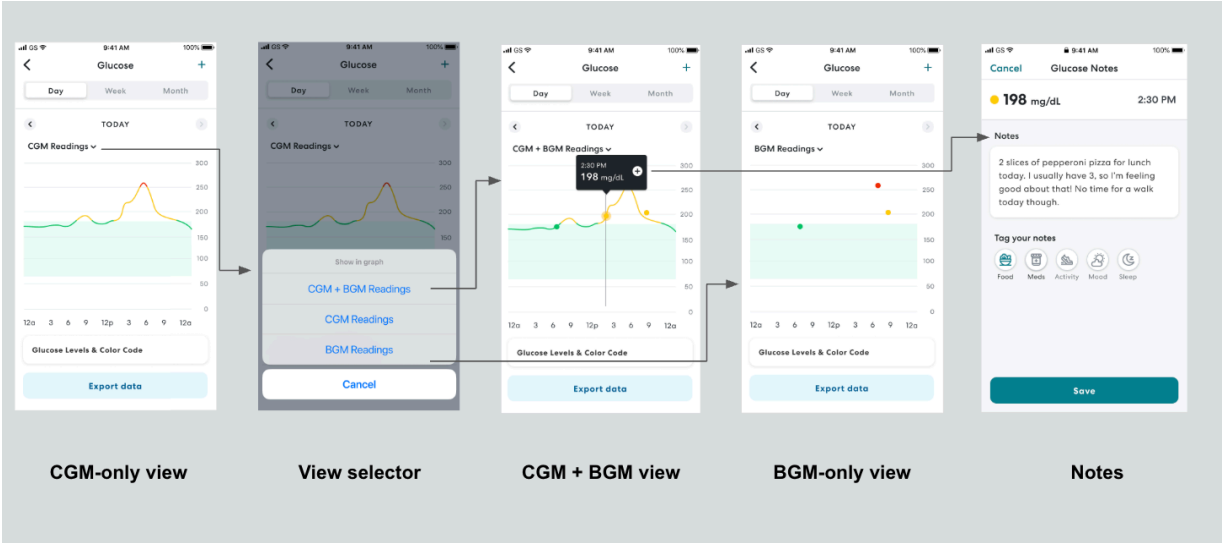


Specialist interaction

Tracking Blood Pressure



Tracking Blood Glucose



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October 1, 2024

The Honorable Micky Tripathi, Ph.D., M.P.P.
Assistant Secretary for Technology Policy
The Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
330 C St SW
Washington, DC 20201
Submitted Electronically

Re: Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability (89 FR 63498, RIN 0955-AA06)

Assistant Secretary Tripathi:

Omada Health, Inc. (Omada) respectfully submits this comment on the Notice of Proposed Rule-making referenced above ("NPRM" or "HTI-2"). For the reasons stated below, we request that the final rule clarify certain aspects of the proposed additions to 45 CFR 171.104.

I. Summary of Argument

Specifically, we request that that the new information blocking standard of omission by "Not exchanging EHI under circumstances in which such exchange is lawful"[1] be further clarified to ensure that the omission of not exchanging is *not* excused by the privately adopted operating rule of Treatment Exchange Standard Operating Procedure (SOP) under Trusted Exchange Framework and Common Agreement.[2] That SOP states that a TEFCA signatory who is a licensed health care professional can refuse to disclose EHI to another health care provider covered entity who is not a licensed healthcare professional, without violating TEFCA. An appropriate clarification might be revise 171.104(a)(9)(I) to state: *Not exchanging EHI under circumstances in which such exchange is lawful, even if the disclosing actor is covered by the ONC Recognized Coordinating Entity Standard Operating Procedure (SOP): Exchange Purpose (XP) Implementation: Treatment and it provides otherwise.* "

II. Omada Health is a "Provider" of "Health Care" under Federal Law, Even If Its Health Care Services Are Not Required To Be Supplied By a Licensed Health Care Professional

Omada is a 13-year-old digital health company that provides care-team-based health care services for the prevention of diabetes and other cardiovascular diseases and for the

[1] 89 FR 63803 proposing new section 9(1) to 45 CFR 171.104).
[2] ONC Recognized Coordinating Entity *Standard Operating Procedure: Exchange Purpose Implementation: Treatment;* https://rce.sequoiaproject.org/wp-content/uploads/2024/07/SOP-Treatment-XP-Implementation_508.pdf.

management of type 1 and type 2 diabetes and hypertension (whether as single conditions or comorbid). We also offer video-based tele-physical therapy. Since our founding, we've served over 1 million adults cumulatively, and we estimate that 20 million individuals have benefits coverage for one or more Omada programs through their employers (including retiree medical plans), health plans, pharmacy benefit managers, health systems, or other customers. Omada is a "health care" "provider" and a HIPAA "covered entity," as defined in federal law, because Omada "furnishes, bills, or is paid for health care in the normal course of business"[3] and meets the other requirements to be a covered entity. For our Diabetes Prevention, Diabetes Education, and Hypertension services, we have been a "provider" "covered entity" since we first offered healthcare services for which we were paid. CMS regulations have also concluded that Diabetes Prevention Program services are the services of a "health care" "provider" and "covered entity" under applicable law.[4] Omada has successfully billed our health plan partners using hundreds of thousands of electronic claims per year, just like any other healthcare provider.

III. Unless ASTP Ensures that the Privately Adopted TEFCA SOP Does Not Override Proposed Regulation 171.104(a)(9)(i), Nationwide Exchange Will Not Be Available to All Health Care Providers "Without Special Effort," as Envisioned by Congress.

As we explained above, Omada is, by law, a health care provider and a covered entity. Federal law makes it lawful for health care provider covered entities to bi-directionally exchange protected health information ("PHI") with each other, without regard to whether or not their services require licensure.[5] Not only is this clear in regulation, but it is clear in educational materials published by ASTP itself.[6] Indeed, the definition of "health care" "provider" applicable to Omada (and many other organizations) has been part of federal law since it was first proposed by CMS's predecessor in 1998.[7] The privately adopted TEFCA SOP, however, states that a query from a provider who is not a licensed healthcare professional does not require a response. This directly contradicts the NPRM proposed addition to 45 CFR 171.104 explaining that conduct can interfere with health information exchange by omission if an actor *fails to exchange when exchange is authorized by law*.[8] The law authorizes exchange among all types of providers, as ASTP's fact sheet cited in footnote 6 above confirms. To ensure that TEFCA signatories who are providers do not interfere with health information exchange by omitting to respond to lawful queries from other providers, the provisions of 171.104(a)(9)(1) must make clear that federal regulation controls when queries require a response, not an SOP put in place by a private entity without notice-and-comment rulemaking. We supplied some suggested language in our Summary section, above.

An example of how Omada engages in health information exchange today illustrates the potential impact of the conflict between the SOP and the NPRM, if finalized as proposed. For an Omada member with diabetes, Omada queries licensed health care professionals to obtain an

[3] 45 CFR 160.103.
[4] 81 Fed. Reg. 80472 (Nov. 15, 2016).
[5] 45 CFR 164.506(c).
[6] https://www.healthit.gov/sites/default/files/exchange_treatment.pdf
[7] 63 FR 25320, 25355 (May 7, 1998).
[8] 89 FR 83803.

individual's baseline hemoglobin A1c ("HbA1c") from physician-ordered lab tests. Omada makes these queries by accessing information from existing health information exchange networks like CareQuality and Commonwell. Omada uses the responses to these queries to obtain baseline data to evaluate an individual's success in managing their diabetes and thus to inform the treatment we supply. In exchange, we disclose back to those providers certain glucose metrics from members' use of continuous glucose monitors. In this transaction today, Omada queries for HbA1c results from tests ordered by licensed health care professionals even though Omada itself is a health care provider and covered entity that is not required by law to use licensed healthcare professionals to supply diabetes self-management services.[9]

This is an issue not just for Omada but for any entity legally defined as a healthcare provider but not required to be licensed. Durable Medical Equipment suppliers and various types of residential programs for seniors come to mind as additional examples.

If the NPRM is finalized as it was proposed *without* language clarifying what constitutes an omission for TEFCA signatories, it will not be clear that TEFCA signatories *must* respond to a query from certain healthcare providers to avoid interfering with health information exchange as defined in the proposed 171.104(a)(9)(i). We propose that the rule not leave ambiguities in place which, in practice, would undermine Congress's intent in the 21st Century Cures Act that all providers (not only licensed health care professionals) be able to engage in legal health information exchange "without special effort." We believe you have an opportunity now to remove that ambiguity in a final rule, and by doing so, to continue to foster nationwide health information exchange among *all* health care providers.

IV. Conclusion

For the reasons stated above, we respectfully request that the text of 171.104(a)(9)(i) be amended as stated above. In the alternative, we respectfully request that the ASTP issue a Request for Information under the Administrative Procedure Act so that it has a well informed basis, with information transparently collected from interested stakeholders, to finalize as rapidly as possible a further update to the information blocking rules that obligates all health care providers to respond to legally authorized queries from any other health care provider, with equal footing.

Respectfully submitted,

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Chief Privacy & Regulatory Officer
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[9] The relevant qualification, Certified Diabetes Care and Education Specialist, is a credential rather than a license in most cases. Only two states (Indiana and Kentucky) currently require CDCES to maintain a state-issued license.